Paclitaxel + cetuximab every 4 weeks

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 8 · paclitaxel only

Active medicines: Paclitaxel

Emetogenic risk: low

Medicine	Dose / route	When
Chlorphenamine	10 mg intravenous (IV)	30 minutes before paclitaxel NCCP Table 7.
Famotidine	20 mg IV	30 minutes before paclitaxel May be omitted only according to consultant guidance and reaction history.
Dexamethasone · conditional	No routine Day 8 dose in NCCP Table 7	Add only if increased risk or previous hypersensitivity, under consultant guidance This differs from the steroid required before cetuximab on Days 1/15. A rechallenge steroid schedule is a separate specialist decision.
Low-risk antiemetic reference	Ondansetron 8 mg orally once on the treatment day if selected	Before treatment under the agreed plan NCIS low-risk reference; individualise QT risk and other medicines. Any rescue medicine and its duration must be prescribed.

Additional phase-specific support

- Arrange nutrition, weight and swallowing review early. Dietitian and speech and language therapy input should follow the person's needs; swallowing safety cannot be inferred from a general leaflet.
- Use the agreed mouth-care and dental plan. Report painful swallowing, mouth ulceration, weight loss or inability to drink before the next appointment.
- Reconcile prescription, over-the-counter and complementary medicines. Discuss fertility, contraception and breastfeeding using the actual products and individual circumstances.
- Confirm the hospital's urgent contact and fever instructions. Infection, bleeding, severe diarrhoea, dehydration, chest pain or new breathlessness need prompt assessment, not the next routine review.
- Review neuropathy and previous paclitaxel reactions. Day 8 is a paclitaxel-only visit within the same 28-day cycle, not maintenance or a different course.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00696 v2 · pNCCP 00696: treatment and supportive-care sections; NCIS v8 pp4–5 where used · NCCP 00696 and current NCIS/local supportive-care guidance

Sources checked 2026-11-10 · SACTCheck v0.84.47 · Regimen-level drafts; prescriber and oncology-pharmacy verification required